ICH-CAHPS

Patient Experience Analysis

A Comparative Analysis of DaVita and
U.S. Dialysis Organizations

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Project Overview

Business Question & Objective

Business Question

How does DaVita's patient experience performance compare to other U.S. dialysis organizations, and where are the largest opportunities for improvement?

Objective

Use publicly available CMS ICH-CAHPS data to evaluate DaVita’s patient experience performance across key domains, identify areas of relative strength and opportunity, and provide data-driven insights to inform potential next steps.



Data & Analytical Approach



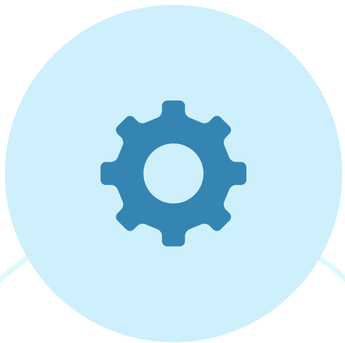
Data Source

- CMS ICH-CAHPS (Dialysis Facility) publicly available dataset (Release: July 2026).



Analytical Population

- Of 7,490 dialysis facilities, 2,594 (34.6%) had reportable ICH-CAHPS results. Performance comparisons were limited to these facilities.



Methodology

- Calculated unweighted averages of facility-level Linearized Scores.
- Compared DaVita with Fresenius Medical Care as the primary benchmark.
- Examined score differences across six patient-experience domains.



Domains Analyzed

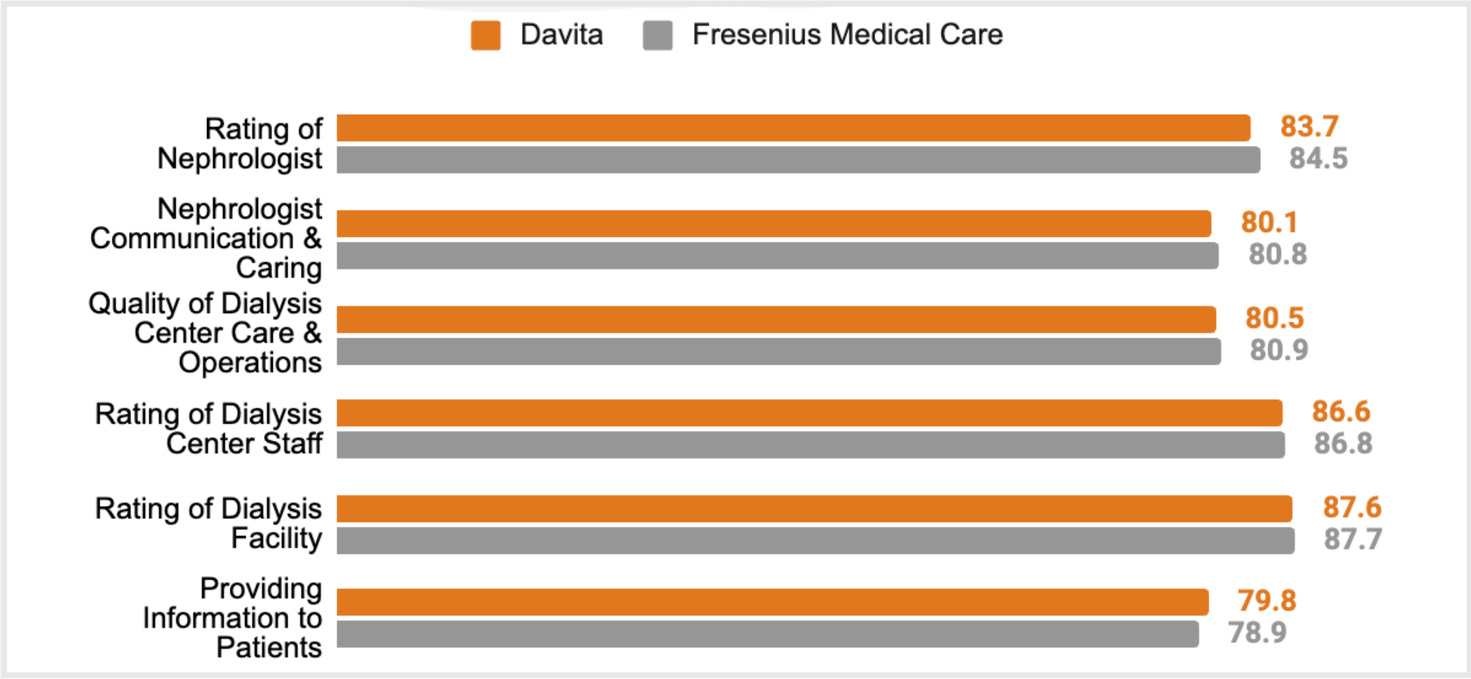
- Nephrologist Communcation & Caring
- Quality of Dialysis Center & Operations
- Rating of Nephrologist
- Rating of Dialysis Center Staff
- Rating of Dialysis Facility
- Providing Information to Patients

STAKEHOLDER SUMMARY

ICH-CAHPS PATIENT EXPERIENCE ANALYSIS

DaVita
PUBLIC DATA ANALYSIS
CMS ICH-CAHPS

DaVita closely tracks Fresenius across all six domains, tailing slightly in five and leading in Providing Information to Patients.



KEY TAKEAWAY

DaVita's patient-experience performance is generally close to Fresenius Medical Care across the six ICH-CAHPS domains.

DaVita trails Fresenius by 0.1-0.8 points by five domains, with the largest observed gap in Rating of Nephrologist. Providing Information to Patients is the exception, where DaVita leads Fresenius by 0.9 points.

INTERPRETATION/ OPPORTUNITY

Rating of Nephrologist shows DaVita's largest observed gap relative to Fresenius (0.8 points), making it a reasonable starting point for further investigation.

However, the differences between the two organizations are small overall, suggesting that DaVita's patient-experience performance is closely aligned with Fresenius across most domains.

CONTEXT

871 DaVita reportable facilities

1,155 Fresenius Medical Care reportable facilities

Scores represent unweighted averages of facility-level Linearized Scores among facilities with reportable ICH-CAHPS results.

Data: CMS ICH-CAHPS (Dialysis Facility)
Release: July 2026

Key Findings & Implications

01. **Performance is closely aligned overall.**
DaVita trails Fresenius bv 0.1-0.8 noints in five of six domains, indicating similar patient experience performance across most areas.

02. **Rating of Nephrologist has the largest gap.**
DaVita's rating of Nephrologist is 0.8 points lower than Fresenius, the largest observed difference among the six domains.

03. **Area of relative strength.**
DoVita outperforms Fresenius in Providing Information to Patients by 0.9 points.



What This Tells Us

- **DaVita's patient experience performance is generally competitive with Fresenius across the six ICH-CAHPS domains.**
- **Nephrologist-related patient experience warrants further investigation, particularly the Rating of Nephrologist domain.**
- **Providing Information to Patients is DaVita's only relative strength against Fresenius, although the results do not establish the reasons for this difference.**

Recommendations & Next Steps

Recommendations

01 Investigate factors contributing to DaVita's lower Rating of Nephrologist score, including potential differences in patient experience across facilities..

02 Examine practices associated with Providing Information to Patients to understand DaVita's relative strength and identify potential lessons for other domains.

03 Conduct a more detailed facility-level analysis to identify specific areas and facilities for targeted improvement.

Limitations

- Only 34.6% of facilities had reportable ICH-CAHPS results, limiting generalizability.
- Unweighted facility-level averages are used.
- Results show associations, not causation.
- Does not include patient-level characteristics or internal operational data.

Next Steps

01 Conduct deeper analysis of Nephrologist domain drivers.

02 Examine performance trends across multiple reporting periods, if comparable data are available.

03 Explore integration with internal data (e.g. staffing, patient mix) to better understand drivers of patient experience.